

DeBakey V.A. Vascular Surgery Discharge Checklist

Timing	ALL discharges: Done by the inpatient team's intern	Additional for "Complex" Discharges ¹ : Done nurse practitioner, case manager, social worker.
	☐ Confirm the patient has a discharge location with adequate physical and social support and a planned mode of transportation to get there.	
Day BEFORE discharge	Medication Reconciliation ☐ Review the dose and duration of new medications with the senior resident/fellow. Determine which (if any) should be continued on an outpatient basis. This is especially important for antiplatelets, anticoagulants, and pain medications! Bowel regimen and DVT prophylaxis meds are often not continued. ☐ Review discharge medications with Clinical Pharmacist via Teams or ext 224180. ☐ Order outpatient medications that are new as of this hospitalization. ☐ Refill existing medications that the patient states he needs. NOTE: Do <u>NOT</u> draft or generate the discharge instructions until AFTER the discharge medication orders are entered. If you do, the discharge instructions will be inaccurate and will cause confusion.	Home Health Services ☐ Nurse practitioner will order wound vacs ≥ 1 days prior to planned discharge and delivered to room. Case manager will follow up with vendor. ☐ Home health wound care: NP and case manager will work with the team to write chart note detailing wound care instructions and frequency. ☐ PT/OT if recommended
	Return to Clinic (RTC) Orders Default vascular surgery follow-up interval is <u>2 weeks</u> . Written by: Orders tab--> RTC Main Menu--> OCL Clinics --> Vascular "HOU VAS SURGERY POST-OP 1653A" if surgical incision or ≥ 8 Fr access. "HOU PHONE VASCULAR SURG" for everyone else. Do NOT overbook unless instructed to by an attending surgeon.	Homemaker/Home aid ☐ Submitted once SW completes case mix identifying qualification for needs.
	Additional clinic follow-up Patients actively followed by the vascular medicine or inpatient medicine teams. ☐ Instruct patient to contact PCP's office for an appointment within two weeks. ☐ Include a statement in the most recent progress note that explains the need for PCP follow-up.	

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	☐ Add the PCP and Care Manager (found by clicking on CPRS tile with PCP name) as cosigners to this last progress note ☐ Order specialty follow-up as needed (examples: ID, cardiology, hematology)	
Day OF discharge	Generate the Discharge Instructions Notes tab --> New Note --> "DISCHARGE INSTRUCTIONS TO PATIENT" Include generic and procedure-specific instructions that are on the GitHub site.	Beneficiary Travel Requests ☐ Case Manager, Social Worker, and Nurse Practitioner will initiate and coordinate/
	Confirmation Confirm plans for discharge with senior vascular trainee or attending surgeon.	
	Discharge order Goal is to enter this before 11am. Orders --> Surgery Orders --> Surgery Orders --> "Discharge Patient with Planned Readmission" (if planned within 30d) vs. "Discharge patient" (for everyone else)	
	Write and sign the discharge summary Use the journal entry-type text from the last progress for the hospital summary.	

¹ Patients discharged with significant needs that are NEW as of this hospital stay, including: discharge destination different from where the patient came from (ex. skilled nursing facility); home health nursing; wound VAC or other dressing changes that cannot be done by the patient or a member of his household; outpatient physical therapy; or outpatient parenteral (intravenous) antibiotics. NOT included: new oral medications.